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A participatory action research qualitative thematic analysis of young men's perspectives on well-being and flourishing

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Abstract

Children constitute 40% of all forcibly displaced people globally. They often face severe mental health challenges due to displacement factors. While evidence-based clinical interventions exist, international demand for mental health services frequently outweighs capacity. This qualitative study explores the interrelationship between individual mental health (flourishing) and broader social and systemic determinants of mental health, highlighting alternative supports for young people's mental health. Using arts-based methods within a Participatory Action Research approach, we collaborated with 22 young men (aged 15–18) living in Atlantic Canada who came from conflict-affected backgrounds, including Syria, Sudan, and Somalia, to explore well-being and its contextual supports. Findings reveal an interconnected web of formal and informal supports, synthesized into four themes: (1) the external bedrock of support, (2) arenas for growth, (3) cultivation of personal growth, and (4) well-being. Findings show that a strong foundation of external support is the prerequisite for the internal growth necessary for flourishing. Crucially, youth highlighted how systemic barriers, such as unsafe public transportation and a lack of accessible neighbourhood spaces, often undermine access to these supports. Results demonstrate that supporting mental health requires a dynamic interplay among the external environment, relational factors, internal development, and policy and implementation supports. Collectively, these findings emphasize the importance of accounting for social and structural determinants of mental health across municipal and government sectors. Here, findings offer actionable implications for policy and practice, particularly regarding structural determinants of mental health, such as the provision of safe, accessible community infrastructure to promote flourishing among refugee youth.

1 Introduction

Children (i.e. people under the age of 18; <https://www.unicef.org/child-rights-convention/convention-text-childrens-version>) account for 29 per cent of the world's population, but 40 per cent of all forcibly displaced people [1]. Furthermore, it is estimated that



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there are currently about 49 million forcibly displaced children globally [1]. The political, economic and social factors that force people into displacement are largely characterized by oppression, marginalization, danger and violence [1]. Children are especially vulnerable to the impact of these factors, often resulting in heightened mental health challenges requiring effective supports and interventions [2]. While there is growing evidence around clinical interventions to support refugee children and youth [3, 4], concomitantly, there is evidence that internationally, demand for mental health services outweighs professional and systemic capacity to respond to this need [5]. Accordingly, we are compelled to consider alternative ways of supporting the well-being of these young people. As Legerski et al. (2024, p. 213) contend, while evidence-based interventions and targeted therapies are essential, “there is a need for more research to address the complex, dynamic interrelationships between individual mental health and broader systemic factors.”

Research focused on the social and structural determinants of mental health shows that contextual and relational supports situated within children’s lived environments can have a profound impact on their well-being outcomes [6, 7]. Given their positioning, these factors are likely to have a much broader reach than clinical interventions. Considered through a dual continuum model [8], we can also surmise that support factors linked to social and structural determinants of mental health will, in some instances, delay the onset of more chronic mental illnesses and, where such illnesses exist, reduce severity and improve prognosis.

Ensuring that impactful contextual and relational supports are situated within young people’s environments, however, requires the direct input of the children and youth (i.e. people between the ages of 15–24; <https://www.un.org/en/global-issues/youth>) living in that context. Their experiences are not merely anecdotal; they constitute critical data essential to the development of effective community programs, social policies, and support systems. By centering their perspectives, we move beyond assumptions that reflect knowledge and engage with the complex, nuanced realities that shape their well-being.

This article reports on research grounded in the principle that those most affected by community conditions are the foremost experts on their own needs. Using arts-based methods within a Participatory Action Research (PAR) approach, we collaborated with a group of 22 young men (aged 15–18 at the start of the study) living in an urban context in Atlantic Canada to explore what well-being entails and the contextual supports that underpin it. Findings highlight an interconnected web of formal and informal supports, embedded within systemic factors that affect accessibility, relevance, and a sense of safety in accessing these supports.

In the following sections, we contextualize this work by reviewing existing research and theory, and then detail the methods used to collect and analyze data. Following this, we present the core thematic findings: the external bedrock of support, the cultivation of an inner resilient self, and the tangible arenas where growth occurs. Finally, we will synthesize these themes into a cohesive model of well-being constructed by the participants as co-researchers, concluding with actionable implications for policy and practice.

2 Literature review

2.1 Supporting youth mental health outside of a clinical context

2.1.1 *Dual continuum model for understanding of mental health and flourishing*

The UN [9] Sustainable Development Goals call for human flourishing, defined as “all human beings can fulfill their potential in dignity, equality, and in a healthy environment.” Similarly, the WHO [10] defines mental health as “a state of mental well-being that enables people to cope with the stresses of life, realize their abilities, learn and work well, and contribute to their community. It has intrinsic and instrumental value and is a basic human right.” Additionally, they note that “Mental health exists on a complex continuum, which is experienced differently from one person to the next. At any one time, a diverse set of individual, family, community and structural factors may combine to protect or undermine mental health.”

Keyes [11], in his work on flourishing over almost three decades, provides a helpful framework for understanding this so-called “complex continuum.” In summary, the Dual Continuum Model of Mental Health comprises two axes, one reflecting mental well-being and the other mental illness. According to the model, an individual can have good or bad mental well-being (reflected on the vertical axis) while also having or not having a mental illness (reflected on the horizontal axis). Flourishing occurs when a person is situated high on mental well-being with a corresponding absence of mental illness symptoms [12]. Subsequent research with the model suggests that promoting mental health and well-being plays an important role in the presence or absence of mental illness symptomatology. Much as good physical health is associated with a lower likelihood of catching a cold and, when a person does get sick, with faster recovery, good mental health is associated with reduced mental illness symptoms and improved prognosis [8, 12]. To this end, drawing attention to health-promoting factors, this model implicates the social and structural determinants of mental health in our efforts to support the mental health of refugee youth.

2.1.2 *Social and structural determinants of mental health*

A substantial body of research emphasizes that mental health outcomes are inextricably linked to the conditions in which people live, learn, and develop. These factors, the social and structural determinants of mental health (SDMH), generate and perpetuate intergenerational cycles of advantage and good health or disadvantage and poor health [7]. SDMH are the contextual conditions – or social determinants – to which individuals are exposed throughout their life course, from conception onward, which influence mental health outcomes and contribute to population-level mental health disparities. These conditions are significantly influenced by economic policies, social norms, and political systems – structural determinants – underpinning the contexts in which people live [13, 14].

Structural determinants are the overarching, upstream factors often considered the root causes of health inequities [15]. They encompass the large-scale political, economic, and social policies, practices, and values that operate at national and local levels. Structural determinants shape the distribution of money, power, and resources, which in turn determine individuals' lived contexts and their opportunities for safe, secure, and healthy lives [7]. Power structures often manifest along class, race and sexual lines, with historical trajectories.

Social determinants, by contrast, represent people's daily lived experiences resulting from the underpinning structural conditions [15]. Economic factors linked to social determinants include financial insecurity, employment status, food insecurity, and housing instability or disadvantage [7, 13, 14, 16]. Education is typically associated with educational inequality, literacy, and parental education levels [7, 13, 14]. Access to support services is often associated with health and healthcare, focused on access to and quality of healthcare, including preventative care and barriers to accessing such care [7, 13, 14, 16]. Social and community context encompasses experiences of discrimination, social isolation, and the presence of social support and community cohesion [7, 13, 14, 16, 17]. Finally, the neighbourhood and the built environment include the physical quality of housing, neighbourhood safety, environmental conditions (e.g., air and noise pollution, lack of green space), and residential segregation or marginalization [7, 13, 14, 16]. The core principle unifying these domains is that differences in SDMH are not randomly distributed, leading to poor mental health outcomes that are disproportionately experienced by individuals living in communities that are socially and economically marginalized [7, 17].

The influence of SDMH is particularly critical during adolescence, when complex cognitive and affective neural systems are being refined and are highly sensitive to the physical and social environment in which this occurs [6]. Being raised in poverty is associated with structural and functional brain changes, often affecting neurocognitive performance [6]. Negative and prolonged contextual factors, such as socioeconomic disadvantage, can accelerate brain development and reduce plasticity. Exposure to environmental factors common in disadvantaged urban areas, such as air and noise pollution and limited green space, are associated with adverse brain development and low mental health in children and adolescents [6, 7].

Additionally, adolescents exposed to adverse SDMH demonstrate significantly higher vulnerability to a range of mental health issues. Poverty is a fundamental risk factor for emotional, mental, and behavioural disorders in youth [14]. Children growing up in socioeconomic disadvantage are two to three times more likely to experience mental health problems compared to their non-disadvantaged peers [7]. Contextual stressors are shown to exacerbate their vulnerability to illnesses such as depression and anxiety [14]. Moreover, poverty exacerbates stress for parents and is consistently linked to child maltreatment and neglect [7].

Critically, when considering the relationship of SDMH and refugee populations, exposure to neighbourhood disadvantage and crime is associated with elevated PTSD, anxiety, and depressive symptoms in adolescence [17]. The highest rates of mental health concerns, including suicide attempts, suicidal ideation, and non-suicidal self-injury, have been found in youth groups categorized by high overall or high social SDMH deficits (adversity and discrimination) [16]. Experiences of discrimination and marginalization translate directly into adverse mental health outcomes. Racial and or ethnic discrimination is related to negative outcomes across multiple developmental domains, with particularly strong effects seen in adolescence [6]. Structural racism also increases exposure to individual-level stressors, such as family conflict and material hardship, which in turn have a negative impact on the brain [7].

Ultimately, the goal of understanding SDMH is to recognize that poor mental health outcomes are not randomly distributed but arise from inequitable access to resources,

often stemming from systemic discrimination and structural inequalities [7, 13]. Addressing youth mental health effectively requires moving beyond individual clinical solutions to implementing broad public health policies that tackle the root causes, such as poverty alleviation and structural racism, particularly during critical early life and adolescent developmental periods [7, 16]. Without addressing structural determinants such as poverty and neighbourhood conditions, and establishing the fundamental conditions necessary for healthy development, a continued focus on downstream individual treatment and intervention will fall short of the needs of refugee youth [6, 7, 15].

2.2 Children and youth as refugees

As previously stated, approximately 49 million children globally are currently forcibly displaced [1]. Many of these children are resettled in host countries as refugees, with or without their families. For example, in 2024, Canada admitted 76,685 refugees and protected persons as permanent residents, of which approximately 17% were minors [18]. These numbers continue to rise annually, reflecting the growing international humanitarian crises [1].

Factors that drive people towards asylum claims are primarily rooted in safety concerns. When people feel structurally unsafe (i.e., their broader social context and government are unable to provide safety) and are confronted with ongoing danger, they are inclined to flee. Key push factors include violence, conflict, and insecurity caused by armed conflict or civil war, ethnic, religious, or political violence, characterized by widespread human rights violations [1]. Additionally, persecution or the threat of persecution, in which a person fears harm because of who they are or what they believe, is another significant push factor. People may be targeted for political opinion, religion, race or ethnicity, gender or sexual orientation, membership in a particular social group (e.g., journalists, activists), and/or tribal or caste identity [1]. Other reasons include state repression or state-sponsored harm, gender-based and family-based violence [19], national economic collapse linked to insecurity (such as extreme poverty caused by war), environmental disasters combined with state failure, and the targeting of children [1, 19], amongst others. The implication is that most people arriving in a host nation as refugees will most likely have had extreme adverse experiences.

Refugee youth experience many of the same mental health concerns as other young people, but the frequency, intensity, and complexity of their needs are often far greater because of exposure to violence, loss, displacement, and resettlement stress. Refugee youth often experience exposure to war, violence, bombing, militia/gang threats, loss of family members, dangerous migration journeys and or detention or family separation. Consequently, they often require support for anxiety and depression, where chronic fear, uncertainty, and disrupted daily life contribute to generalized anxiety, panic symptoms, sadness and hopelessness, and sleep disturbances and nightmares [20, 21]. Mental health challenges are then often augmented by uncertainty stemming from aspects of the resettlement process. These include immigration hearings, family reunification, housing instability, and financial insecurity [22, 23]. Strengths that these youth may bring with them, such as creativity, strong insights, cultural, familial and community bonds, and multilingual and adaptive capacities, are often undermined by these negative experiences.

In response, youth require stabilization through a sense of safety and predictability. Environments characterized by consistency, predictability, clear routines and provision of basic needs foster and support mental well-being. The absence of these exacerbates mental health challenges [23, 24]. Additionally, refugee youth often navigate two worlds at once. Managing this requires culturally responsive counselling, interpretation and language support, space to affirm identity and cultural practices, and community-based, relational supports rather than purely clinical ones [23, 25]. For refugee youth, schools are major sites of stress and support. Here they need peer integration support, anti-racism and anti-bullying supports, trauma-informed teachers, academic catch-up assistance and support for interrupted education [23, 26]. Refugee youth also require support for acculturation stress. Youth face pressure to adapt quickly, role reversal with parents, identity conflict and discrimination or xenophobia [2, 27–29]. To manage this, they often need identity development support, spaces to belong (youth groups, mentorship), and guidance in navigating multiple cultural expectations [30, 31]. Finally, and importantly, they need their own strengths recognised and supported [24]. What these support factors look like locally however, and how they interact with one another, is largely contextually shaped. Understanding these needs in broad terms does not mean we understand the specificities of these supports and their nuanced functioning within specific communities, for specific populations.

2.3 This study

Findings presented in this article are from a study exploring the contextual factors that support youth well-being in an urban area of Atlantic Canada, Spaces&Places for Youth Wellbeing. Situated within a social and structural determinants of mental health model, the study responds to the uptrend in the rates of young people struggling with their mental health, unable to experience well-being [32]. Specifically, we sought to collaborate to identify mid- and upstream mental health supports [33], centring youth experiences in the results used to inform policies focused on mental health promotion [33]. The study was conducted in partnership with a local YMCA, the Community YMCA who intentionally engages in activities to work with hard-to-reach youth. Youth researchers in the study all identified as young men and were predominantly from Syria, Sudan, and Somalia (aged 15–18 at the start of the research). These countries are identified by the UNHCR [1] as being characterized by violence, conflict, and insecurity caused by armed conflict or civil war, ethnic, religious, or political violence, characterized by widespread human rights violations. The youth had been in Canada for an average of three years prior to the study, and all are living with at least one of their parents and their siblings. All youth are living in socially and economically marginalised sectors of the city.

3 Research methodology

Spaces&Places for Youth Wellbeing used qualitative arts-based methods within a participatory action research (PAR) framework, with 22 youth to unpack what well-being looks like to young men from conflict backgrounds, and what factors in their lived environment support and or jeopardize their well-being. Youth were informed of the study by staff at the Community YMCA. Where they were interested, youth were invited to a meet-and-greet session in which the study and their research rights were explained to them. Following this, several sessions were held, during which the team of student and

youth researchers got to know one another through informal activities. Sessions were facilitated by the study's lead researcher (Liebenberg).

Following this, data gathering began. We first made use of a photo-elicitation process [34], where youth were asked to photograph, over seven days, the spaces they move through in the city and to reflect on how these spaces support or undermine their well-being. This process was intended to facilitate deeper reflection on taken-for-granted experiences in their lived environment [34]. Once images had been made, individual interviews were conducted with the youth to explore their photos in depth, expanding the young men's thinking about well-being factors in their lives [34]. We then held a focus group meeting where the youth shared their photos and key interview findings using flip chart pages converted into analogue facebook pages. Through the sharing of their photos and themes from their individual interviews that they felt were important to the research, the discussion amongst the participants, was further enhanced, bringing diverse and collective perspectives to the discussion [35].

We then worked with the youth in three smaller groups to explore emerging factors in more detail. Here, we used various facilitation tools. We used body mapping to explore in more detail what well-being looks like and entails. Youth drew the outline of a body on large flipchart pages, and discussed in small groups what a healthy happy person looks like while filling in the details of their "person." The group discussion served to bring reflections from their individual interviews together and deepen their reflections on well-being. The information was then shared back to the larger group for a larger group discussion, again enriching the understanding of well-being. They then arranged large cards with previously identified contextual factors written on them, around their happy health person to examine in greater depth the interaction between youth and their lived environment. Again, this process served to incrementally enrich their perspectives and the related data on upstream mental health supports. These methods were intentionally chosen for their unique capacity to explore the rich context of personal experience.

Finally, the youth worked with the lead researcher and students to analyze the data using thematic analysis [36]. Here, youth worked with code cards reflecting the codes that had emerged from their interviews, focus group, and workshop sessions, sorting them through a consensus building process, into groups. During this process, the youth explained how they understood the code, why it mattered to the research, and why it belonged with a certain group of other codes. These groups were then further discussed to identify the themes represented by these code groups, as well as the relationships between the themes. By collectively constructing a narrative model of well-being from individual concepts, the youth directly engaged in the analysis process. This approach ensures that the resulting framework accurately reflects their shared understanding and priorities. This process also fosters researcher reflexivity in several ways. First of all no one individual is responsible for the analysis. Done within a group setting, biases and personal subjectivities are accounted for. Second, as the analysis process unfolds, all youth researchers have the opportunity to challenge or affirm findings. The following sections present the core themes that emerged from this collaborative, participatory analysis, reflecting the building blocks of well-being as defined by the youth themselves.

The transcript of the analysis workshop conducted with the youth participants forms the basis of the findings section. As such, little to no excerpts from transcripts are included as the findings section reflects the analytical voices of the youth themselves.

Rigour was maintained in various ways throughout the project. First, using multiple data sources (i.e. individual interviews together with small and large group discussions), allowed the team to cross-verify findings. Second, reflexivity was maintained by checking in with the youth throughout the research process, that the academic research team (LL & TS) correctly understood what the youth were sharing, and that the analysis reflected youth experiences. This provided an opportunity to identify any personal biases and preconceptions that could be entering into understanding the data. Third, youth engagement in the data analysis and dissemination process ensures that what is shared accurately reflects participant experiences. Finally, the voracity of youth engagement is furthered by the prolonged engagement as a team. We worked together for nine months on data gathering in addition to the time spent developing relationships at the beginning and the time spent on data analysis and dissemination at the end.

Ethical consent was obtained from the Health Sciences Research Ethics Board of Dalhousie University, the host institution for the research. In accordance with this, all youth provided freely given, informed written consent to participate, and where necessary, due to age, from their guardians.

4 Findings: the building blocks of well-being

Findings reflect a web of interconnected personal, relational, and contextual supports that interact to promote young people's well-being. While conceptualized as a web, there is still an anchoring foundation of supports that youth need to experience the personal growth required for well-being. Building on these foundational supports are additional critical components, including education, culture, and other contextual resources, that interact to foster personal growth and ultimately well-being. These findings echo research on the SDMH, extending them to provide a contextualized understanding of how these determinants interact to support well-being (see Fig. 1).

4.1 The bedrock of support: community context and relationships

Our analysis revealed that a supportive external environment is the essential foundation for individual well-being. Youth researchers consistently emphasized that before a young person can focus on internal growth and personal achievement, they require a secure and nurturing base. This foundation is built on positive relationships with family, friends, and the broader community, which together foster a sense of safety and belonging.

4.1.1 The role of family and community

Youth identified the concepts of “safe families and communities” and a “sense of belonging” as deeply interconnected. They articulated a two-directional model where community is defined not just as a physical place but as a web of relationships that “helps families be more supported. And the more families are supported, the better the community is.” This reciprocal dynamic creates an environment where individuals feel secure and valued enough to engage meaningfully with their surroundings. However, this foundational bedrock is frequently threatened. Youth contrasted the ideal of safety with the lived reality of environments where they see “needles around the place like the parks,” roads near vital community hubs where shootings have occurred, and public spaces like the library where drug use necessitates a call for greater security. Ultimately, if the physical environment is not safe, a sense of belonging (as bolstered by relationships) is

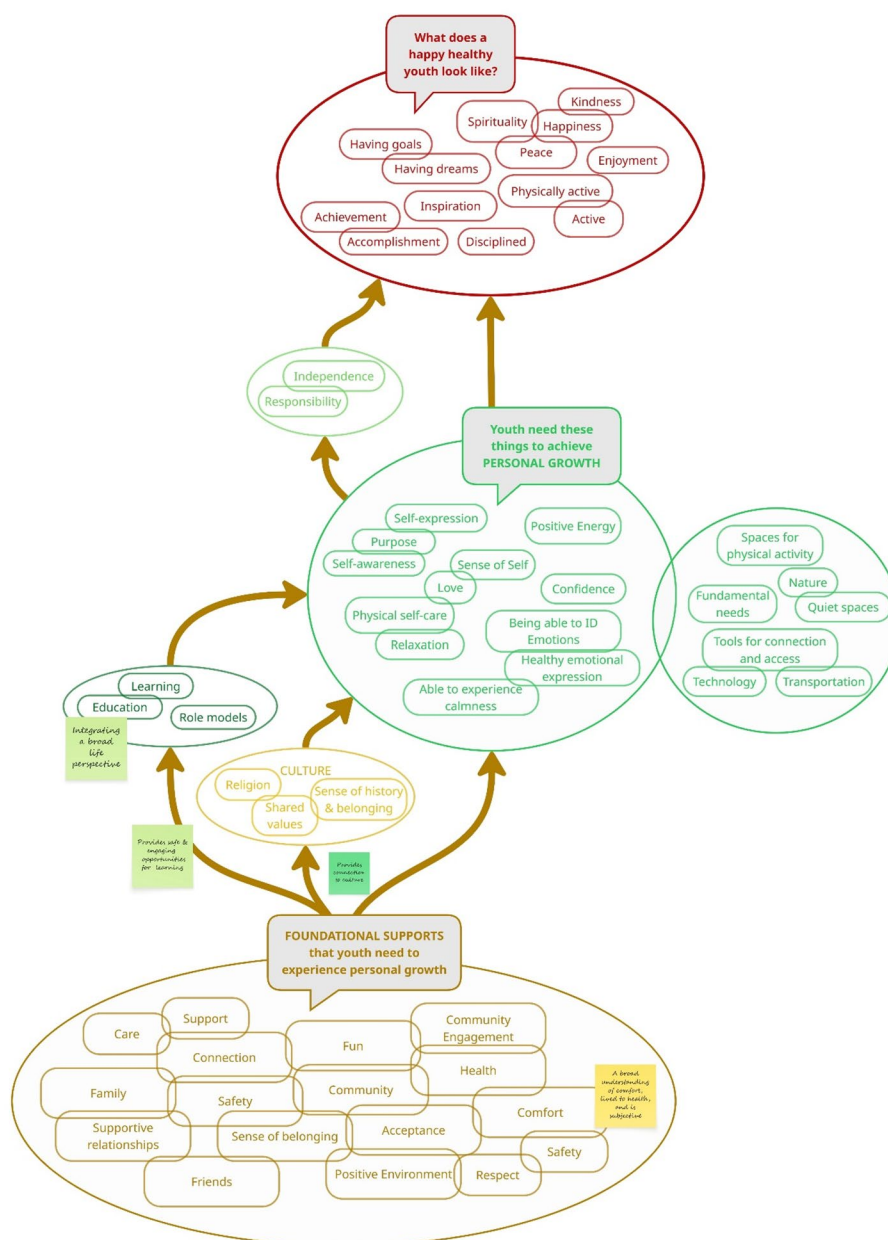


Fig. 1 Codes and themes of youth wellbeing

undermined. By contrast, a safe environment creates space for families and community members to express care and have fun. Youth also explained that a safe physical and relational environment is characterized by a sense of acceptance, mutual respect, comfort (a subjective sense of acceptance for who you are and being okay), and being “healthy.”

4.1.2 The function of friendships

Friendships were identified as a distinct and vital pillar of support. Youth stressed the importance of “healthy friendships, where we experience acceptance, comfort.” While family support is crucial, friends offer a unique, relatable perspective built on shared experience. As one youth noted, with friends, “you go through the same things and can help each other because you understand.” This suggests a relationship of mutual

understanding and peer-based validation that differs from the often-hierarchical support of family, especially where parents have come from such a different social context to the one they are living in now. This peer connection provides a crucial space for acceptance among those navigating similar life stages, but also seems to solidify relationships with family because of shared youth experiences with parents. This external foundation of community and relationships provides the security necessary for young people to turn inward and develop their personal attributes.

4.2 Arenas for growth: key spaces, activities, and tangible needs

The research demonstrated that abstract concepts of well-being become concrete in the tangible settings of daily life. This section explores the specific places, activities, and material needs that Youth identified as crucial arenas for their happiness, health, and personal development. These arenas are where the internal skills of the “Inner Architect” (see next section) are cultivated and tested.

4.2.1 The significance of structured community spaces

Specific community hubs were repeatedly cited as vital to youth well-being, though access is not always guaranteed. For the youth in this study the Community YMCA is valued for providing food and safe, fun activities, and was described as a welcoming place that feels like “family.” Youth noted however, that limited operating hours restrict their access around school hours. Similarly, the mosque serves as a critical space for spiritual and community connection. Youth described it as a place that provides peace, shared purpose, and a sense of belonging, making it a key environment for emotional regulation and self-reflection. Finally, the library was identified as a safe, quiet place for study, personal development, and social connection. Like the mosque, it functions as an arena for achieving the “calmness” and focus needed for personal growth.

In addition to providing welcoming and safe spaces, all three venues offer youth engaging opportunities for learning. Youth explained how the learning that occurs in these spaces is completely different to the form of pedagogy used in school spaces, yet they found this learning so much more impactful and meaningful. Learning in these spaces occurs through both active engagement and observation of others (role models). Additionally, through a process of reflection after doing, by having conversations with older role models, youth can analyze and understand the cause-and-effect component of experiences and their related success or failure. They highlighted the importance of this learning to their own growth and ultimate well-being.

Again, these findings highlight the rich interaction between safe physical community spaces, and the impact of supportive relationships within these spaces. Here, youth especially noted how these physical and relational spaces also facilitated their access to and relationship with culture, both their families’ cultures and the new culture in which they are now living.

4.2.2 The role of sports and physical activity

Sports and physical activities, particularly soccer and basketball, were consistently framed as essential for well-being. Spaces like the BMO Soccer Facility are more than just fields; they are primary arenas where the internal skill of discipline is cultivated and a sense of accomplishment is earned. Youth explained that beyond physical health, these

activities foster teamwork, build social connections, and serve as a healthy escape and a positive outlet for energy.

Importantly, they also highlighted that while formal spaces are needed for more formal engagement in these activities, access to informal spaces is equally important. Informal spaces could be more accessible by being situated within their own neighbourhoods and being free for use. To this end, the youth shared how they had previously been able to spontaneously meet up and play soccer in an open lot in their neighbourhood before it had been turned into an apartment complex. Now they have nowhere to do this. Additionally, to get to free spaces in the city, they have to rely on public transportation. They are reluctant to do this for several reasons. First, it removes the spontaneity and ease with which they can gather, requiring more planning. Second, by leaving their communities, they have to go into other city spaces where they may not feel as welcome or safe. And finally, they are hesitant to use public transportation because of feeling unsafe. This is because of the racism expressed by bus drivers, the lack of safety from other passengers and the dirty buses. Relatedly, they noted that these factors often mean it is easier (and safer) to default to their phones.

4.2.3 The importance of nature and quiet spaces

Connected to informal outdoor spaces, youth articulated a strong need for “quiet places” and “green spaces.” Youth spoke of the calming and restorative effects of being in nature, linking these spaces directly to the practice of emotional awareness. These environments provide a necessary escape from stressful surroundings and are seen as essential for self-care, relaxation, and achieving the sense of “peace” required for self-reflection.

Additionally, however, green spaces provide sites of connection with friends, and a space for families to connect and spend time together. Here youth noted the presence of play parks in their communities, but also discussed how these spaces were too small to accommodate activities like community gatherings or adolescents kicking balls around. By contrast, they argued that spaces where families could connect as part of a broader community served to strengthen social fabric, interpersonal understanding and connection, and ultimately intergenerational relational supports and establish a sense of belonging and safety.

4.2.4 Tools for connection and access

Interwoven through their identification of core support factors was the pragmatic role of resources that make connections with relationships and physical spaces possible. Most notably, they spoke of cell phones as a means of communicating with friends and family, learning about activities, and accessing transportation services such as Uber and the bus. Along with this, however, they also noted the drawbacks of resources such as public transportation. As previously noted, the youth highlighted that the social and physical conditions of buses make them reluctant to use public transportation.

Crucially, access to all these arenas for growth is often obstructed by systemic barriers. Youth highlighted unreliable public transportation—including infrequent service, dirty conditions, and experiences of racism on busses—as a significant obstacle preventing them from reaching the very places that support their well-being.

Similarly, when they don’t feel a sense of belonging and engagement, including times when they cannot spontaneously connect with friends in their neighbourhood (linking

back to safe, accessible green spaces), they are more inclined to scroll on their phones. Youth also explained that when phones and online spaces are used for recreation, they are used at times when “we need to escape or there is nothing to do.” Youth noted that when they feel overwhelmed by life, or are unable to connect face-to-face with friends, that is also when they will go online, especially to participate in games (like FIFA).

4.3 Cultivating personal growth

When external foundational supports are available, youth identified a powerful set of internal characteristics and practices that they are able to engage with and that are essential for navigating life’s challenges and achieving personal goals. These internal attributes foster the personal growth required to build personal strengths, a sense of purpose, and achieve emotional stability, as well as ultimately, a sense of well-being.

4.3.1 Tangible expressions of self-care

Youth discussed the importance of tangible elements like “nice clothes,” a “fresh haircut,” and “good hygiene.” They explained that this is not about materialism but expressions of “self care” and “self pride.” Taking care of one’s appearance was directly linked to feeling “mentally happy” and confident. These practices represent a form of self-respect that contributes positively to one’s internal state.

4.3.2 Emotional awareness and regulation

The ability to understand and manage one’s own emotions was identified as a critical skill for inner well-being. Youth noted skills like “identify your emotions” and “express your emotions” as prerequisites for achieving “calmness” and “confidence.” The logic, as one youth articulated, is that “you won’t be able to stay calm because you don’t know why you’re lashing out.” This underscores the importance of emotional literacy as the basis for self-control and stable, positive emotional states. These internal traits are not developed in a vacuum; they are nurtured and expressed within the tangible spaces and activities that make up a young person’s daily life as reflected in the foundational components.

Self-awareness is also linked to emotional literacy as well as foundational components. Being able to interact with others, engage in diverse activities, have varied experiences, and understand how these experiences make you feel interacts to support self-awareness, healthy self-expression, and develop a sense of purpose in life. They also interact to build a sense of self and confidence.

Collectively, these factors also come together to foster a bi-directional sense of responsibility and independence. Here youth explained that healthy personal growth, fostered by foundational supports, as well as education and connection to culture, can instill a sense of responsibility for self, others and community. These factors also foster personal independence. Critically, the combination of the two ensures that while young people grow to be self-supporting, they are not selfish in their actions. To be self-supporting in healthy ways, requires recognising your co-dependence on others, and the need to be equally responsible for yourself and others. This overall cluster of ideas highlights the belief that self-development is an active, ongoing process that builds identity and direction.

4.4 Achieving well-being: a happy, healthy self

Discipline was viewed as a cornerstone concept that “ties in with everything else.” Youth described it as a proactive, positive force necessary for personal growth, achieving goals, and maintaining positive relationships. One youth connected it directly to accomplishment, stating, “if you don’t have that discipline, you’re not going to have the accomplishments.” This view extended beyond personal achievement to the relational sphere, with another participant noting its practical necessity in maintaining connections: “If you take care of your relationships once a month, you’re probably not going to be in relationships.” This theme positioned discipline as the fundamental practice enabling the consistent effort required for a flourishing life.

Relatedly youth who are flourishing, have dreams and goals that are intended to realise these dreams. Dreams are varied and connected to school and work, relationships and community, as well as recreation and fun. Youth articulated a clear and dynamic relationship between having a “sense of self,” from “personal growth,” and having “dreams and goals.” They saw these concepts as feeding into one another in a continuous cycle. As one individual explained, “the more you know, as you’re growing the more you know yourself.” This growing self-knowledge, in turn, fuels a sense of purpose and the ability to set and pursue meaningful goals. Additionally, youth who are flourishing have a sense of inspiration; they are inspired to work towards their goals and dreams, drawing on their discipline. Through this, they have a sense of achievement and accomplishment that feeds into a cycle with inspiration, goals and dreams, fostering greater discipline. Youth noted that flourishing doesn’t mean not experiencing failure, not achieving, or getting things wrong. Rather, these seemingly “negative” experiences serve as opportunities to reflect, learn and grow. Such growth, however, is often dependent on relational supports where others can support such reflection and learning. It also requires access to future opportunities to “try again!”

In addition to these characteristics, flourishing is seen to encompass being active and finding enjoyment in life. Importantly, these activities are linked to happiness and kindness towards self and others, but also being able to have a sense of peace because of spirituality. Youth explained that having faith means you can make sense of experiences and events in your life, supporting a sense of happiness and peace. However, when faith is paired with religious engagement, you have a community of people around you who can guide you in your faith and meaning-making, especially when experiences are especially difficult.

5 Discussion

5.1 A holistic model of youth well-being

The findings from this research present a model of what well-being looks like and how it is supported for a group of young men who have come to a mid-size urban setting in Canada, from war-torn contexts. The use of multiple elicitation techniques provided progressively richer understandings of well-being and the upstream, community-based supports needed to support it. The resulting data and analysis yielded rich and nuanced understandings of how mental health and well-being is supported. These methods also facilitated participation among individuals with differing communication preferences and capacities, and strengthened the credibility of the findings through triangulation. Ultimately, the model outlined in the findings section highlights what young men living

with experiences similar to theirs require if their mental health is to be promoted as a buffer for mental challenges and illness [12]. The model's factors reflect what we know about the social determinants of mental health [15], the importance of social safety, integration and belonging [23], and the mediating role of community and peer group support between trauma and depression, especially for young men [37]. However, the model also demonstrates that supporting mental health does not rely on a static list of factors; rather, it is a dynamic interplay among a young person's external environment, their engagement with the world as mediated by relationships, their internal development, and overall well-being. It tells a story of how flourishing is cultivated. The model, as reflected in Figure 1, follows a clear and logical sequence: If we provide young people with supportive relationships, safety, and community, then we provide them with learning opportunities. Collectively, these feed into a process of personal growth that ultimately underpins a happy, healthy young person. This model emphasizes that a strong foundation of external support is the prerequisite for the internal growth that ultimately defines a happy and healthy individual. Echoing what we know about social and structural determinants of mental health, however, social determinants (i.e. contextual and relational supports) are themselves required to be "healthy" if they are going to be positive supports. To be healthy, they require supportive structural determinants (i.e., municipal and government policies and their implementation that account for their impact on mental health) [13, 15]. In this way, these findings also hold important implications for the structural determinants of mental health.

Key aspects highlighted by the youth was the importance of spaces in which they can engage with social connections, friends especially, in spontaneous and convenient ways. There are key ways in which this data omits and or deviates from a more generic understanding of SDMH. First youth did not discuss economic factors. That said, much of what they did discuss implicates parental emotional and time availability (such as engagement in community events). Such availability requires fiscal stability. Similarly, housing stability and health of homes and the surrounding built environment are required to support cognitive development, physical health, and achievement of goals and dreams. Second, while most Western models of SDMH ordinarily associate education with more formal education systems, young people are increasingly explaining how education is so much more than this [38, 39]. Here too, youth in this study refer to the complex interplay of relationships, opportunities to try things and get it wrong, and lived environment in their educational outcomes. These insights expand our understanding of what education means to young people, informing not just what healthier formal education systems could look like, but also the diverse actors and contexts that should be included in these systems. Similarly, the youth in this study highlight aspects of town planning and infrastructure (the built environment) that require consideration. Public transportation, for example, not only needs to be available, but also needs to be clean and safe. Such a requirement is probably not only relevant to this group of youth, but many other groups within urban centres, such as people who identify as female, professionals, parents with young children, and the elderly. Importantly, safety doesn't only concern the behaviour of other passengers but also the behaviour of bus drivers and their interactions with passengers. Finally, having green spaces and recreational facilities within cities is not enough. Thought needs to be given to where these spaces are positioned, and who has access to them. Accessibility in turn is not just about cost, or location, but also

size and resourcing. The young men in this study were very concerned about the discomfort or even harm they could cause by just playing with a soccer ball in their local “park” because it was small and filled with play structures for younger children. When the importance of green spaces, face-to-face social connection, and engagement in physical activity to fostering mental health vis-à-vis depression, anxiety and PTSD in adolescent refugee populations [15, 17] are considered, the importance of these considerations becomes clear.

Our findings offer a distinct perspective on the well-being of young men that both complements and diverges from other established frameworks like the Socio-Ecological Model [40, 41], and the UNHCR MHPSS pyramid [42]. While many sources focus on mitigating pathology and risk, the YMCA model emphasizes personal growth and the proactive cultivation of a “happy, healthy self”. Our findings are grounded in the social and structural determinants of mental health (SDMH), a concept shared with other models focused on socio-ecological factors and social determinants. Like the social-ecological models presented by Scharpf et al. [40, 41], our findings highlight nested levels of influence, moving from family and community contexts to individual attributes. Both sets of findings agree that well-being is not a static list of factors but is achieved via a dynamic interplay between a person’s external environment and their internal state. Interestingly, while other SDMH models, such as that of Sim et al. [43], highlight economic stressors like housing instability and immigration loans as primary drivers of distress, this youth-led model omitted direct discussion of economic factors. Instead, the participants prioritized relational and spatial supports, such as safe green spaces for activities like soccer. This is not to undermine the critical importance of economic factors, but to understand the importance of the supports that manifest from economic supports (like parental emotional availability and safe green spaces).

The Social Participation Framework presented by Niemi et al. [44] focuses on refugees’ access to and agency within host societies. Both models highlight how host country policies and regulations (Dimension 1 in the Social Participation model) determine the extent of possible engagement. Our findings again demonstrate how this manifests by highlighting, for example, how the characteristics of public transportation create structural barriers, noting that racism and safety concerns on buses prevent youth from reaching essential support spaces. Relatedly, the geographic positioning of recreational spaces augments these barriers. The dynamics and characteristics of all these factors are underpinned by policy. By contrast, however, important nuanced differences exist between these models. While the Social Participation model categorizes engagement into formal roles (student, employee) and community groups, our model emphasizes spontaneous and informal social connections that reflect youth in their totality. It argues that for youth, the ability to meet friends conveniently in their own neighbourhood is more vital for belonging than formal institutional involvement.

Perhaps most importantly, are the ways in which our findings align and differ with the UNHCR MHPSS pyramid [42] a service-delivery framework that ranges from basic services to clinical care. The emphasis in our findings on foundational supports (i.e. safety, community, and health) align with the lower layers of the MHPSS pyramid, which focus on basic services and strengthening family/community supports. Unlike the MHPSS pyramid, which places clinical mental health services at the top tier, our findings highlight the critical importance of informal community supports that sit outside a clinical

context. Our findings advocate for “mid- and upstream” supports that may delay the onset of chronic illness through strong social supports and contextual resources.

Importantly, the findings of this study have their limitations. This research has been conducted with a group of 20 youth who identify as male in a mid-size urban setting. The voices of female-identifying youth, and the diversity of youth who identify as LGBTQAI+, need to be accounted for in future research. Additionally, quantitative survey results may be used to affirm the resonance of these findings with larger populations of youth. Similarly, the insights of younger adolescents are absent from these findings and should also be explored in future research. Recognizing that the Community YMCA intentionally works with marginalized, silenced and hard-to-reach youth, selection bias may still exist where those youth most isolated in the city are still not reached.

Additionally, the various approaches to qualitative data gathering used in this study create the potential for variability in the type and depth of data generated across techniques. We believe, however, that if present, this limitation was accounted for in the analysis with the young men, allowing for the integration of data derived from different modes of expression. And while such variability may influence interpretation of the findings, we again believe that this was countered by the collaborative analysis and dissemination process.

6 Conclusion

This research has detailed a model of youth well-being that is complex, interconnected, and deeply rooted in the lived experiences of 22 young men who have come to a mid-size urban Canadian context from backgrounds of extreme adversity and violence. The primary finding is that well-being is not supported by a simple checklist of resources, but engages a dynamic ecosystem of relevant and accessible supports. It is built upon a non-negotiable foundation of safe and supportive relationships within families and communities. This secure base—when it can be established against real-world threats—enables young people to cultivate internal strengths, purpose, and self-awareness through meaningful engagement in safe, accessible, and purposeful activities and spaces. These activities cultivate the internal growth that youth need in order to flourish, not just cope, with the complex challenges they have experienced that threaten their mental health and well-being. These relational and contextual supports, however, require policy and related resource allocation from local and national leadership in order to both be present and persistent.

The findings of this study are unique in their focus on young men and their understanding of well-being and required supports. Through their own deep reflection, they have identified tangible expressions of well-being, the interconnected web of supports they require for this well-being and the nuanced interaction of these supports in relation to self in the journey to well-being. Most critically, our intent was not necessarily to develop a new model, but rather to explore the nuances of which social determinants are relevant in the lives of young men in urban spaces. Similarly, we sought to illustrate the interactive nature of these components, as required to support wellbeing.

Author contributions

LL is the lead researcher on the study. She led the data gathering, analysis and writing of this article. TS is the project manager and assisted with the data gathering, analysis and writing of this article. EE is a co-applicant on the project and assisted with key revisions and final drafting and editing. TJ is the lead of the community partner and assisted with recruiting participants and the final drafting and editing of the article. YR are the youth researchers on the study who

participated in the data gathering and analysis. The findings section is written from the transcript of the data analysis workshop.

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Data availability

The data that support the findings of this study are available from the lead author, but restrictions apply to the availability of these data, given the ethics approval of the research from the host organization, and so are not publicly available. The data are, however, available upon request, subject to the approval of Dalhousie University's Research Ethics Board.

Declarations

Ethics approval and consent to participate

The Dalhousie University Health Sciences Research Ethics Board approved this study. The research was carried out in accordance with the Canadian Tri-Council guidelines. Freely given, informed consent to participate was obtained from all participants, and, where necessary due to age, from their guardians.

Consent for publication

Freely given, informed consent to publish findings from the study was obtained from all participants, and, where necessary due to age, from their guardians.

Competing interests

The authors declare no competing interests.

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